

Introduction

Research suggests that there is a relationship between hormonal patterns across the span of a day and conditions including sleep disorders and depression. In this study, the focus is on the hormones, DHEA and Cortisol, which play a role in regulating stress. Cortisol is an important steroid hormone that significantly affects metabolism which in turn affects the central nervous system. DHEA is

In this study, researchers followed the cortisol and DHEA patterns over the course of a day for a total study period of three days to measure the patterns of the two hormones in a natural environment.

Researchers utilized two methods to collect saliva samples; a Saliva Procurement and Integrated Testing (SPIT) booklet and an automatic Electronic Monitoring Cap. Saliva samples were collected at waking, 30 minutes after waking, before lunch, and 600 minutes after waking. Subjects did not have to adhere to a specific wake up time for 31 healthy control subjects.

This report addresses the following hypotheses of interest:

Hypothesis 1: There was satisfactory agreement between the subject's recording of sampling times compared to the recording times by the electronic monitoring cap. More specifically, there is a one-to-one relationship between the subject's recorded times and the booklet times.

Hypothesis 2: Subjects adhered appropriately to the study protocol of the +30 minute and +600 minute sampling times.

Hypothesis 3: DHEA and Cortisol levels show an initial increase from waking to 30 minutes followed by a gradual decline from 30 minutes and on.

We will further refer to the electronic monitoring cap as MEM time and the the SPIT Booklet as Booklet time for the remainder of the report.

Methods

Preliminary Data Analysis

Below is a Table1 of the primary variables of interest for the analysis:

Table 1: Characteristics of Study Cohort

	Overall	Wake Time	+30 Min	Pre Lunch	+600 Mi
n	372	93	93	93	93
Minutes Since Waking (Midnight) (mean (SD))	235.78 (262.28)	0.00 (0.00)	33.49 (8.22)	321.89 (84.42)	642.09 (85.42)
Booklet Time (Min Since Midnight) (mean (SD))	654.35 (272.68)	415.91 (86.85)	448.09 (85.46)	736.34 (71.54)	1056.89 (111.54)
MEM Time (Min Since Midnight) (mean (SD))	684.24 (279.26)	417.58 (88.41)	463.51 (89.78)	734.99 (68.44)	1068.24 (121.54)
DHEA (nmol/L) (mean (SD))	0.98 (1.03)	1.78 (1.27)	1.11 (0.92)	0.51 (0.46)	0.50 (0.61)
Cortisol (nmol/L) (mean (SD))	5.95 (6.95)	7.38 (6.05)	9.00 (4.94)	3.21 (2.29)	4.15 (10.3)

For analysis, we exclusively worked with the time variables in minutes. Looking at Minutes Since Waking, we can see that the mean minutes closely follow the study protocol times. The average DHEA and Cortisol levels appear to be similar across the 4 time points throughout the day.

As missingness can present serious issues for the data analysis if not addressed, we investigated missingness through calculating the number and percentage of missing values for each variable in the dataset. We further visualized missingness for each variable.

For the variables we employed in our analyses, we assessed and visualized distributions to determine if there were any outliers or considerations that needed to be taken for data analysis.

Data Cleaning

For DHEA, we removed observations that were ≥ 5.205 (nmol/L) as these are considered extremely high DHEA levels that could interfere with analysis. There were 6 observations (4 of which corresponded to Subject 3037) that had DHEA levels of 5.205 (nmol/L). We removed all observations for Subject 3037 due to multiple high DHEA values that suggest underlying health problems as well as the 2 other observations that had DHEA of 5.205 (nmol/L).

For cortisol, we observed 5 observations from 4 patients that had Cortisol levels ≥ 26 . Although these values are considered high, they are biologically plausible and they were left in the analysis. One observation had a cortisol level of ~89 which was excluded for analysis.

Data Analysis

To address the first hypothesis, we employed a longitudinal linear mixed effects model with Booklet time in Minutes for the outcome with covariates MEM in Minutes and the day number (1-3). Adding Day as a covariate noticeably improved diagnostic plots for the assumptions of heteroscedasticity and normality. We included a random intercept for subject that allowed us to take variability between subjects into account in our model.

To determine whether subjects were adhering accurately to the +30 minute and +600 minute sampling times required by study protocol, intervals of time were defined to determine the number and percentage of patients that had good or adequate adherence. Good adherence was defined by a collection of the saliva sample within a 7.5 minute window interval whereas adequate adherence if samples were within a 15 minute window. To calculate the percentage and proportion of individuals that had good adherence and adequate adherence, the difference between the “correct” sampling time and the observed

time was calculated and this difference was used to assess whether observed times were within the good or adequate adherence windows.

We visualized and summarized the distribution of the MEM and Booklet times to guide us in our decision to proceed with either MEM or Booklet times for our calculations. We observed that MEM times had a considerable amount of missingness and determined that reporting both would give a better overall sense of adherence. We summarized adherence for the study population with tables.

For the third hypothesis, we used a longitudinal linear mixed effects models to assess whether DHEA and Cortisol levels followed the expected diurnal pattern. Due to a right-skew in the distribution of DHEA, a log transformed DHEA variable was used as the outcome to assess DHEA changes. No transformations was made to Cortisol. Covariates for oboth models include manually calculated pre and post 30 minute variables which would give us the slopes for the two time intervals of interest.

All analyses were conducted in R version 4.4.1 (2024-06-14), using the nlme package for modeling.

Results

From the missingness investigation, we determined the MEMs had the highest amount of missing: 177 missing values which is equivalent to 47.6%. Booklet had the second highest amount of missing with 55 missing values or 14.8%. Cortisol and DHEA measurements had 5 missing values or 1.34% missingness.

Hypothesis 1

Below is a table of the estimates, 95% Confidence Intervals and P-values for the model:

Table 2: Model Summary

	Value	Std.Error	DF	t-value	p-value
(Intercept)	3.436	4.355	41	0.789	0.435
MEM_C	0.986	0.010	41	102.912	0.000
factor(DAYNUMB)2	-3.148	1.550	41	-2.030	0.049
factor(DAYNUMB)3	-1.642	1.558	41	-1.053	0.298

Holding day constant, a one-minute increase in MEM-recorded wake time is associated with an approximately one-minute increase in Booklet-recorded wake time ($\beta \approx 0.99$; 95% CI: (0.97–1.01); $p < 0.0001$). This near one-to-one association indicates strong concordance between the two wake-time measures within a given day. In other words, the subject's recorded wake up time and the MEM's recorded wake up time track each other quite closely. Changes in MEM time are mirrored almost exactly by changes in Booklet time and our initial hypothesis is correct.

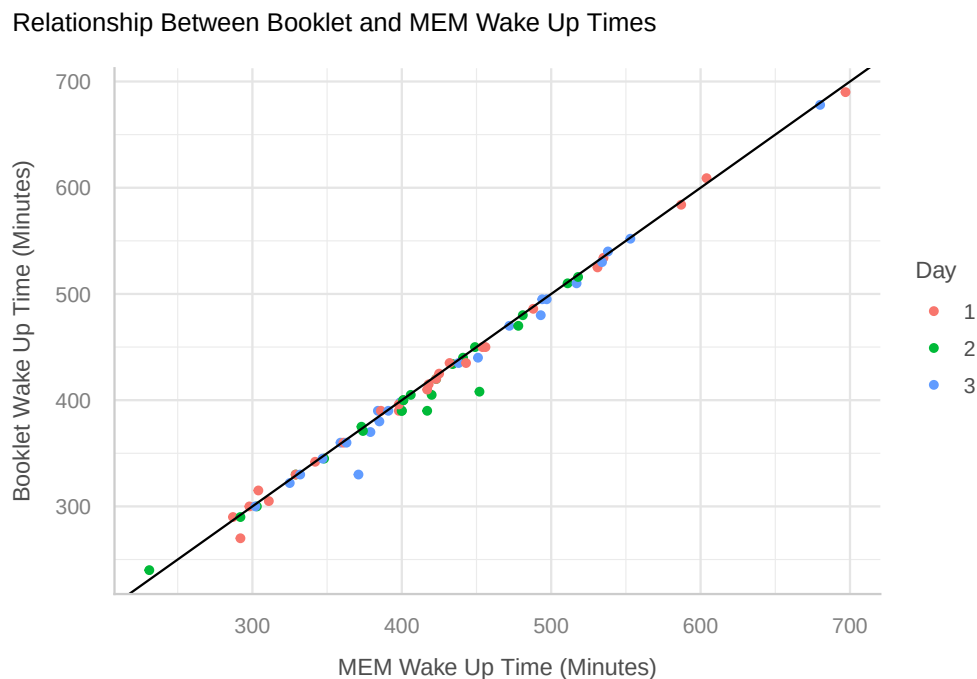


Figure 1: Booklet Vs. MEM Wake Up Times

We additionally assessed bias by.... COME BACK TO THIS

Hypothesis 2

Below we summarize the percentage of subjects that adhered to the study sampling times. Percentages for the +30 Minute sampling time is in Table 2 and percentages for the +600 Minute sampling time is in Table 3, for both MEM and Booklet times.

Table 3: Percentages for Good and Adequate Adherence by Collection Window

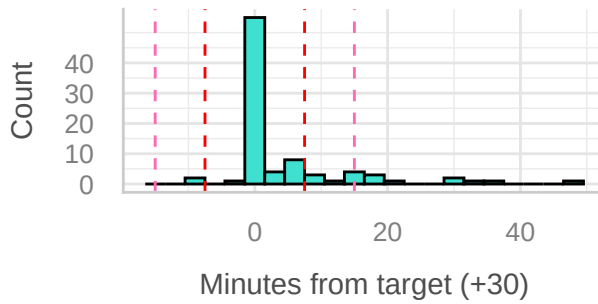
Window	Method	Good	Adequate
+30 minutes	Booklet	78.16	89.66
+30 minutes	MEM	52.86	71.43
+60 minutes	Booklet	46.25	56.25
+60 minutes	MEM	32.53	39.76

Overall, more subjects are meeting the adequate adherence thresholds compared to the good adherence thresholds for both the +30 minute and +600 minute sampling times. In addition, the MEM percentages are lower for both sampling times and for both good and adequate thresholds.

Below we visualize the minutes from the +30 minute sampling time (left column) and the +600 minute sampling time (right column). Times for Booklet are in the top row and time for MEM are in the bottom row. The red dashed lines indicate the ± 7.5 window for good adherence while the pink dashed lines are the ± 15 minute window for adequate adherence.

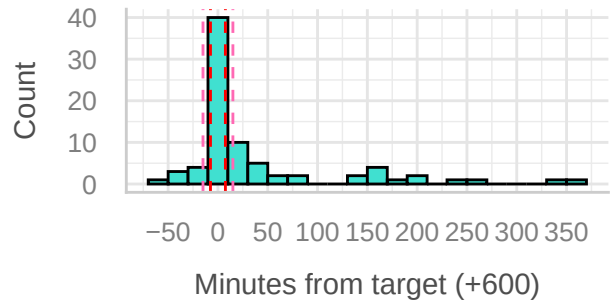
Minutes from +30 Minute Sampling Time

Booklet



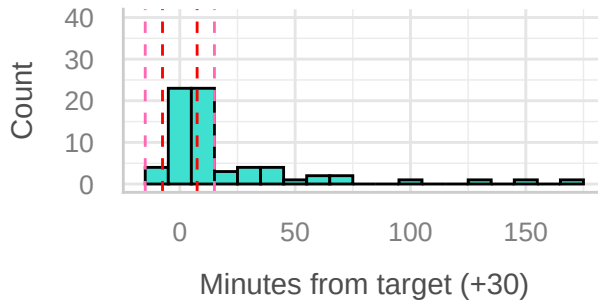
Minutes from +600 Sampling Time

Booklet



Minutes from +30 Minute Sampling Time

MEM



Minutes from +600 Minute Sampling Time

MEM

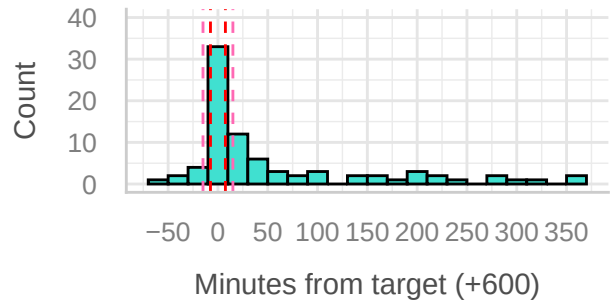


Figure 2: Adherence for +30 and +600 Sampling Times

Hypothesis 3

```
## Warning in .parse_hms(..., order = "HM", quiet = quiet): Some strings failed to
## parse
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## parse
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## parse
## Warning in .parse_hms(..., order = "HM", quiet = quiet): Some strings failed to
## parse
## Warning in .parse_hms(..., order = "HM", quiet = quiet): Some strings failed to
```

Table 4: Model Summary

	Value	Std.Error	DF	t-value	p-value
(Intercept)	0.21392	0.11741	288	1.82195	0.0695
time_pre30	-0.01724	0.00301	288	-5.73083	0.0000
time_post30	-0.00153	0.00016	288	-9.87455	0.0000

```
## parse
```

```
## Warning in .parse_hms(..., order = "HM", quiet = quiet): Some strings failed to
```

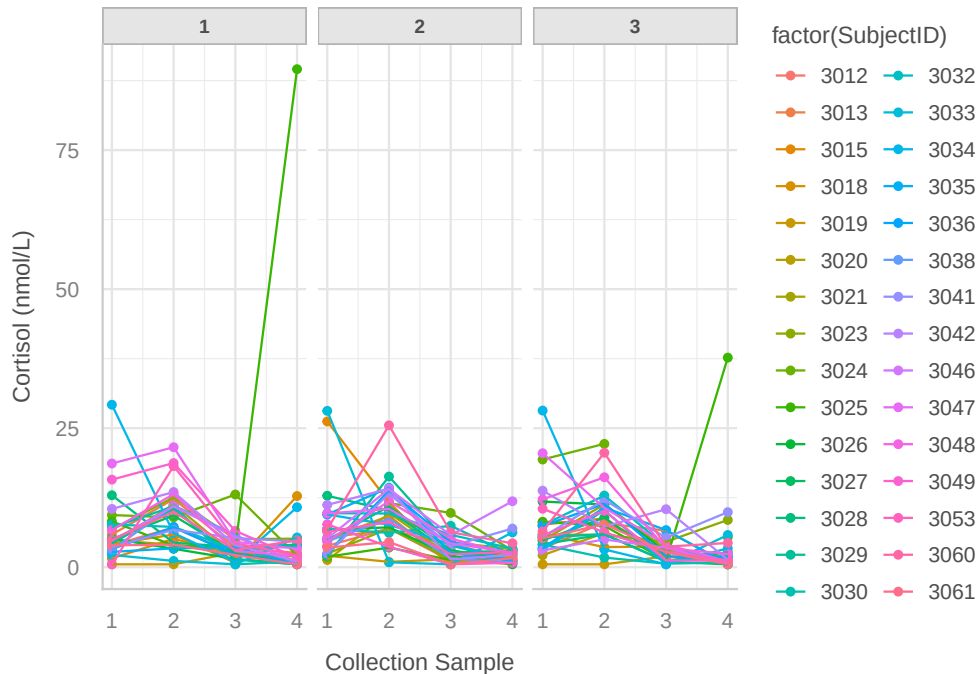
```
## parse
```

Interpretations:

Time Pre30: For the time period of wake time to 30 minutes, there is approximately a 40% decrease in DHEA (95% CI: (-50.04%, -28.75%)). This is statistically significant with $p < 0.001$. DHEA shows a statistically significant decrease, not the expected increase, from waking to 30 minutes after waking.

Time Post30: From 30 minutes to 600 minutes, there is approximately a 60% decrease in DHEA levels (95% CI: (-66.65%, -51.91%)). This is statistically significant with $p\text{-val} < 0.001$. As expected, DHEA showed a statistically significant decline over 10 hours.

Using a piecewise linear mixed-effects model with a knot at 30 minutes post-waking, DHEA concentrations showed a significant early decline - approximately 39% decrease over the first 30 minutes. After 30 minutes, DHEA declined more gradually across the day, corresponding to an additional ~60% decrease over 10 hours.



Interpretations:

Time Pre30: For the time period of wake time to 30 minutes, there is approximately a 54% increase in Cortisol (95% CI: (-60.19%, 503.15%)). This is not statistically significant with $p = 0.53$. Although there is an increase from waking to 30 minutes after waking, it is not a statistically significant increase.

Time Post30: From 30 minutes to 600 minutes, there is approximately a 99% decrease in DHEA levels (95% CI: (-99.86%, -97.27%)). This is statistically significant with $p\text{-val} < 0.001$. As expected, Cortisol showed a statistically significant decline over 10 hours.

Cortisol follows the expected pattern of an increase from waking to 30 minutes followed by a decline from 30 minutes and on.

Discussion